

MATERIALS NEEDED

IPG (Implantable Pulse Generator)

Supplied in a single use package as a kit with the following configuration:

- One sterile IPG Model 2104 with therapy OFF
- One sterile torque wrench



CSL (Carotid Sinus Lead)

Supplied in a single use package as a kit with the following configuration:

- One sterile CSL – Model 1036
- One sterile Implant Tool



Programmer

Supplied as a kit, in a case, with the following components:

- Programmer software
- Programmer Interface – Model 9020
- Tablet with power cord
- USB-C cable



Model 9020

SURGICAL PREPARATION

Skin Preparation

Follow your institution's surgical skin preparation practices. Consider applying an agent that is effective against typical skin flora.

Consider application of an impermeable skin barrier to minimize contact between the implanted components and the patient's skin.

Antibiotic Coverage

Follow your institution's antibiotic coverage practices. Consider antibiotic coverage in the perioperative period if this is not the current practice. Consider a second dose of antibiotics during the implantation procedure.

Sterile Packaging

The IPG, CSL, and Implant Tool are supplied STERILE (by ethylene oxide gas) and for SINGLE USE. Reuse of this product may result in malfunction, adverse event or death.

The components are supplied in a sterile package for direct introduction into the operating field. Carefully inspect the sterile packages before opening. Do not use if the package is opened, damaged or has evidence of damage or compromised sterility. Return the package and/or contents to CVRx.

Do not open the package if it has been exposed to extremes of temperature outside of the temperature range stated on the labeling, or if there is damage to the package or the package seal. Return the package, unopened, to CVRx.

Do not use on or after the "Use By" date. Return the unopened package to CVRx.

Prior to opening the IPG package, use the Programmer System to check the IPG battery. If the reported battery voltage is less than 2.85V return the package, unopened, to CVRx.

To open the package, do the following:

1. Grasp the tab and peel back the outer cover.
2. Using sterile technique, lift out the inner tray.
3. Grasp the tab on the inner tray and peel off the inner cover to expose the contents.
4. Remove the product.

Device Interaction Testing

If problematic interactions cannot be eliminated at implant, the Barostim System should be explanted. It is recommended that patients are monitored for interaction following implant and whenever changes are made to either device. If device interactions that develop after implant cannot be eliminated, the Barostim IPG should be programmed "OFF". Below are instructions to perform device interaction testing.

- If a patient has a CRM device (e.g., pacemaker, ICD or CRT-D), perform device interaction testing once the IPG is in the pocket and before the patient emerges from anesthesia.
- Ask CRM device rep or hospital device tech to:
 - Maximize CRM device sensitivity. For devices with auto-sensitivity (e.g., ICDs), program the device to pace faster than the intrinsic rate to assure device achieves highest sensitivity.

- If applicable for CRM device, compare morphology discrimination criteria with Barostim off and on.
- Test the recommended Barostim settings:

Pulse Width	Frequency	General Anesthesia	Local Anesthesia or Post-Operative
30 μ s	40 pps	20 mA (or highest within compliance)	
170 μ s	40 pps	12 mA	Maximum before extraneous sensations
500 μ s	40 pps	8 mA	

If any interactions are observed record information in session summary report and mitigate by:

- Restoring CRM device sensitivity to programmed value.
- Lower Barostim amplitude until interaction no longer occurs.

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Implantation Procedure

Surgical approach and techniques for the implantation of the system will vary with the preference of the implanting surgeon. Requirements for proper and safe implantation of the system should include the items covered in this section.

CLINICAL UPDATE

Based on the BeAT-HF PMA Amendment database from April 30, 2019, the implant position for the carotid sinus lead (A-D, picture below) was evaluated for chronic response by position. As shown in Table 1, the results are consistent across lead location (A vs. B-D) supporting the elimination of the requirement to map during the implant. It is recommended that the implant location be prioritized at location “A”. In the instances where implanting at location “A” is anatomically undesirable, choose a location as close to “A” as the anatomy allows, based on the implanting physician experience. In the Intended Use Population of the BeAT-HF, 90% of the subjects had the lead implanted in position A on the carotid artery. Table 1 shows the percent of subjects that had a clinically relevant response at six months in 6MHW (≥ 25 meters), QoL (≥ 5 points) and/or NT-probing ($\geq 10\%$ relative reduction), as well as the percent of subjects who responded to at least one or two of these endpoints. The response rates across all endpoints were similar regardless of the final lead position.

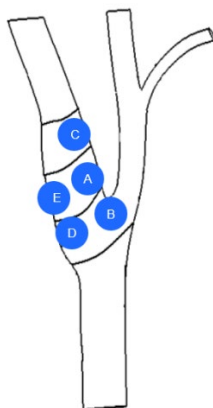


Table 1: Response by Lead Location in Intended Use Subjects

Variable	Lead Location A			Lead Location B-D		
	N	Mean $\pm$ SD or N (%)	Median (Range)	N	Mean $\pm$ SD or N (%)	Median (Range)
6MHW Responder	103	72 (69.9%)	N/A	11	7 (63.6%)	N/A
QoL Response	104	73 (70.2%)	N/A	12	10 (83.3%)	N/A
NT-proBNP	104	58 (55.8%)	N/A	12	5 (41.7%)	N/A
Response ≥ 1	104	99 (95.2%)	N/A	12	11 (91.7%)	N/A
Response ≥ 2	104	77 (74.0%)	N/A	12	9 (75.0%)	N/A